

Viva Dental Prep — Chapters 9 & 10 Clinical Role-Play Dialogues

Interactive Learning Scripts for Spanish-Speaking Dental Hygiene Students

(Chairside Patient Communications & NBDHE Board Question Solutions)

■ ABOUT THESE ROLE PLAYS / ACERCA DE ESTOS JUEGOS DE ROL

These role-play scripts are designed specifically for native Spanish-speaking dental hygiene students to practice **chairside patient communication** while reinforcing NBDHE board exam concepts.

The Chapter 9 scripts feature **real clinical team-to-patient interactions** (Patient, Dental Assistant, Dental Hygienist, and Dentist) to practice what to say directly to a patient during an appointment. The clinical team represents 3 levels of English proficiency:

1. **Dental Assistant (Beginner English)**: Takes vitals, asks basic medical history questions, and clarifies simple terminology.
 2. **Dental Hygienist (Intermediate English)**: Conducts intraoral exam, explains clinical findings to the patient in clear empathetic English, and discusses home care.
 3. **Dentist (Advanced English)**: Conducts doctor exam, diagnoses complex conditions, explains systemic/microbial connections, and highlights NBDHE board concepts.
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■ ROLE PLAY 1: CHAPTER 9 — CASE A (REVISED CHAIRSIDE CLINICAL INTERACTION)

Clinical Setting: Dental Operatory Hygiene Exam & Doctor Consultation

Patient: Mrs. Evelyn Goddard (44 y/o female, HIV+, complaining of severe gum pain and tongue lesion)

Clinical Team Characters:

Ilmary (Beginner English)*: Dental Assistant (taking vitals and medical history).

Nancy (Intermediate English)*: Registered Dental Hygienist (RDH) (explaining periodontal findings to patient).

Dr. Christopher (Advanced English)*: Dentist (confirming diagnosis, systemic connections, and treatment plan).

■ CHAIRSIDE CLINICAL SCRIPT: CASE 9A

Ilmary (Dental Assistant - Beginner): Good morning, Mrs. Goddard! My name is Ilmary. Welcome to our dental clinic. How are you feeling today?

Mrs. Evelyn Goddard (Patient): Hello, Ilmary. Honestly, my gums and teeth hurt a lot. It is very painful when I try to brush or eat.

Ilmary (Dental Assistant - Beginner): I am sorry you are in pain, Mrs. Goddard. Let me take your blood pressure first. Your blood pressure is 128 over 78. Now, let's review your health history. You noted that you are HIV positive, and you take Atenolol for high blood pressure and Zidovudine (AZT) for HIV. Is that correct?

Mrs. Evelyn Goddard (Patient): Yes, that is correct. I have been taking my AZT medication regularly.

Nancy (Hygienist - Intermediate): Hi Mrs. Goddard, I am Nancy, your dental hygienist. Thank you for sharing your history. Because HIV affects the immune system, it can make your gums much more sensitive to infection. Let's look inside your mouth. I see generalized red, swollen gums with areas of dead tissue that bleed easily upon probing.

Mrs. Evelyn Goddard (Patient): Oh dear... why are my gums dying and bleeding like that? Is it just regular gingivitis?

Nancy (Hygienist - Intermediate): No, Mrs. Goddard. This is more advanced than regular gingivitis. Because the tissue shows necrosis—which means dead tissue—and you have bone loss around your teeth, this condition is called **Necrotizing Periodontitis**.

Dr. Christopher (Dentist - Advanced): Good morning, everyone! I am Dr. Christopher. Nancy is completely right. Mrs. Goddard, **Necrotizing Periodontitis** is an aggressive periodontal condition that commonly occurs in patients with immunocompromising conditions, such as HIV. That relates directly to **Board Question #1 & #2**: Necrotizing Periodontitis presents with severe pain, bleeding, and tissue necrosis, and it frequently affects HIV-positive patients due to lowered immune defense.

Ilmary (Dental Assistant - Beginner): Dr. Christopher, I also noticed a white spot on Mrs. Goddard's tongue when I seated her.

Nancy (Hygienist - Intermediate): Yes, Doctor! On the dorsum of her tongue, there is a raised, white patch. I tried to wipe it gently with a gauze square, but it is **nonremovable**—it does not wipe off.

Mrs. Evelyn Goddard (Patient): Is that thrush? Will it wipe off if I brush my tongue at home?

Dr. Christopher (Dentist - Advanced): No, Mrs. Goddard, do not try to scrape it hard. **Board Question #3** highlights this exact lesion: Pseudomembranous candidiasis (thrush) wipes off easily, but a raised white plaque that is **nonremovable** is classified as **Hyperplastic Candidiasis**. It is a fungal infection that requires prescription antifungal therapy.

Nancy (Hygienist - Intermediate): Dr. Christopher, she also has a cavity on tooth #2 on the chewing surface.

Dr. Christopher (Dentist - Advanced): Yes, that is pit-and-fissure decay. Mrs. Goddard, decay in those top grooves is primarily initiated by a bacteria called ***Streptococcus mutans*** (**Board Question #4**). And regarding your HIV history, as we discuss risk factors for our records, HIV is bloodborne and most commonly transmitted through intravenous drug use or blood exposure (**Board Question #5**). We will prescribe an antimicrobial rinse, antifungal medication, and start gentle periodontal therapy to heal your gums!

Mrs. Evelyn Goddard (Patient): Thank you so much, team! I feel so relieved knowing what is happening and how we can treat it.

■ VOCABULARY & PRONUNCIATION GUIDE — CASE 9A

English Term	Spanish Translation	Simple Definition	Pronunciation Guide
Necrotic Tissue	Tejido Necrótico	Dead tissue resulting from infection or loss of blood supply.	<i>neh-KROT-ik TISH-oo</i>
Necrotizing Periodontitis	Periodontitis Necrosante	Severe painful periodontal infection destroying gingiva and bone.	<i>NEH-kroh-tye-zing peer-ee-oh-don-TYE-tis</i>
Nonremovable	No removable	Cannot be wiped or scraped away.	<i>non-ree-MOOV-uh-bul</i>
Hyperplastic Candidiasis	Candidiasis Hiperplásica	Fungal lesion appearing as a white patch that does NOT wipe off.	<i>hye-per-PLAS-tik kan-dih-DYE-uh-sis</i>
Intravenous (IV)	Intravenoso	Delivered directly into a vein.	<i>in-trah-VEE-nus</i>
Pit and Fissure Decay	Caries de Fosas y Fisuras	Cavities forming in the deep grooves of molar teeth.	<i>pit and FISH-er dee-KAY</i>

■ HELPFUL BOARD EXAM TIPS — CASE 9A

[!IMPORTANT]

Clinical & Board Exam Pearl:

- 1. Chairside Patient Explanation:** Explain to patients that "necrotic" tissue means dead tissue requiring special antimicrobial treatment.
- 2. Candidiasis Distinction:** If a white plaque **wipes off** → Pseudomembranous Candidiasis. If it is **nonremovable** → Hyperplastic Candidiasis.

■ ROLE PLAY 2: CHAPTER 9 — CASE B (REVISED CHAIRSIDE CLINICAL INTERACTION)

Clinical Setting: Dental Operatory Examination & Consultation

Patient: Mr. Steve Perry (32 y/o male, history of shingles, Type 1 diabetes, complaining of cracked lip corners)

Clinical Team Characters:

Manal (Beginner English)*: Dental Assistant (taking patient history and vitals).

Ilmary (Intermediate English)*: Registered Dental Hygienist (RDH) (evaluating chief complaint and root caries).

Dr. Nancy (Advanced English)*: Dentist (diagnosing viral history, fungal lesions, and congenital enamel defects).

■ CHAIRSIDE CLINICAL SCRIPT: CASE 9B

Manal (Dental Assistant - Beginner): Good morning, Mr. Perry! My name is Manal. Welcome to our practice. What brings you to see us today?

Mr. Steve Perry (Patient): Hi, Manal. My lips keep cracking at the corners, and it hurts when I open wide. Also, I haven't had my teeth cleaned in a long time.

Manal (Dental Assistant - Beginner): I understand, Mr. Perry. Let's review your health history. You noted that you have Type 1 diabetes and take Metformin. You also have a history of shingles. Is all of that current?

Mr. Steve Perry (Patient): Yes, that is correct. I had a very painful shingles outbreak on my side a few months ago.

Ilmary (Hygienist - Intermediate): Hello, Mr. Perry, I am Ilmary, your dental hygienist. Having diabetes can make healing slower, so keeping your blood sugar balanced with Metformin (**Board Question #5**) is very important for your oral health! Let's examine your lips first. The corners of your mouth are red, cracked, and inflamed. This condition is called **Angular Cheilitis**.

Mr. Steve Perry (Patient): Angular Cheilitis? What causes my lips to crack like that?

Ilmary (Hygienist - Intermediate): Angular cheilitis is a fungal skin infection at the lip commissures (corners). It is caused by an overgrowth of a yeast called **Candida albicans** (**Board Question #2**). We will give you a topical antifungal cream to heal it!

Dr. Nancy (Dentist - Advanced): Good morning, Mr. Perry! I am Dr. Nancy. Let's review your shingles history and dental X-rays. **Board Question #1** addresses shingles: Shingles is a painful nerve and skin outbreak caused by reactivation of the **Varicella-Zoster Virus (VZV)**, the same virus that causes chickenpox in childhood.

Mr. Steve Perry (Patient): Ah, I see! And Doctor, why do my front teeth feel sensitive near the gumline?

Ilmary (Hygienist - Intermediate): Mr. Perry, during your probing exam, I found Class V decay along the gumline on teeth #8 through #11.

Dr. Nancy (Dentist - Advanced): That is root surface decay, Mr. Perry. For **Board Question #3**, while *Streptococcus mutans* causes enamel cavities, Class V root surface decay is primarily caused by **Actinomyces naeslundii** bacteria, which colonize exposed root surfaces.

Manal (Dental Assistant - Beginner): Dr. Nancy, when I took Mr. Perry's dental intraoral photos, I noticed his first molars have a unique shape with many extra small bumps.

Dr. Nancy (Dentist - Advanced): Great observation, Manal! Those are called **Mulberry molars**. **Board Question #4** tests this oral manifestation: Mulberry molars (and Hutchinson incisors) are classic developmental tooth shape malformations caused by **Congenital Syphilis**. We will restore the Class V decay with composite fillings, treat the angular cheilitis, and get your smile healthy!

Mr. Steve Perry (Patient): Thank you, Dr. Nancy, Ilmary, and Manal! I appreciate you taking the time to explain everything so clearly.

■ VOCABULARY & PRONUNCIATION GUIDE — CASE 9B

English Term	Spanish Translation	Simple Definition	Pronunciation Guide
Shingles (VZV)	Hóster / Culebrilla	Painful viral skin outbreak caused by Varicella-Zoster Virus reactivation.	<i>SHING-guls</i>
Angular Cheilitis	Queilitis Angular (Boqueras)	Fungal infection causing cracking and sore redness at corners of the mouth.	<i>ANG-gyoo-ler kye-LYE-tis</i>
Mulberry Molar	Molar en Mora	Molar tooth with multiple berry-like extra cusps from congenital infection.	<i>MUL-beh-ree MOH-ler</i>
Class V Root Decay	Caries Clase V en Raíz	Cavities located along the gumline on root surfaces.	<i>klas FYVE ROOT dee-KAY</i>
Lip Commissures	Comisuras Labiales	The corners where the upper and lower lips join.	<i>KOM-ih-shurs</i>
Topical Antifungal	Antifúngico Tópico	Medication cream applied to skin to eliminate yeast/fungal infections.	<i>TOP-ih-kul an-tee-FUNG-gul</i>

■ HELPFUL BOARD EXAM TIPS — CASE 9B

[!TIP]

Caries Bacteria Etiology Comparison:

* *Pit & Fissure Enamel Decay* → *Streptococcus mutans*

* *Root Surface (Class V) Decay* → *Actinomyces naeslundii*

* *Deep Dentin Caries Progression* → *Lactobacillus*

■ ROLE PLAY 3: CHAPTER 10 — CASE A

Case Subject: Jean, RDH (Autoclave Wet Pouches, Wicking & Sterilization SOPs)

Characters & English Levels:

Ilmary (Beginner English)*: Dental Assistant Trainee (observing sterilization lab).

Jean (Intermediate English)*: Temp Registered Dental Hygienist (RDH).

Christopher (Advanced English)*: Lead Sterilization Educator & Compliance Specialist.

■ DIALOGUE SCRIPT: CASE 10A

Ilmary (Beginner): Hi, Jean! Welcome to Bright Smile Dental Office. I see you are unloading the autoclave sterilizer. But wait... why are those instrument pouches wet?

Jean (Intermediate): Oh no! You are right, Ilmary! The instrument packages feel damp and wet. Another employee opened the autoclave door before the drying cycle finished because she needed extraction pliers immediately!

Christopher (Advanced): Hello Jean and Ilmary. This is a critical infection control issue. Jean, based on **Board Question #1**, what is the **FIRST** thing you must do with these wet instrument packages? Can we just put them in the drawer to dry?

Ilmary (Beginner): Can we dry them with a paper towel?

Jean (Intermediate): No, Ilmary! Wrapping paper becomes porous when wet! Bacteria from the air or our hands can travel through the wet paper into the sterile instruments. We must **re-bag the instruments in new pouches and run a complete new sterilization cycle including the full drying cycle!**

Christopher (Advanced): Exactly right, Jean! That answers **Board Question #1**. Now, **Board Question #2** asks for the scientific name of that process. What is the term when moisture draws bacteria through porous packaging materials?

Ilmary (Beginner): Is it called absorption? Or contamination?

Jean (Intermediate): The board term is **Wicking**! Moisture acts like a wick on a candle, pulling ambient microorganisms through the paper barrier!

Christopher (Advanced): Excellent! **Wicking** is the exact term for **Board Question #2**. Now, let's look at **Board Question #3**: How could our dental office improve sterilization protocols when temporary employees like Jean arrive?

Ilmary (Beginner): Should someone give temp employees a tour?

Jean (Intermediate): A tour is helpful, but the office must have **written Standard Operating Procedures (SOPs)** available for all staff to read!

Christopher (Advanced): Correct! Having written SOPs ensures everyone follows CDC guidelines regardless of temp status. And finally, **Board Question #4**: Using wet instrument packages results in **bacterial contamination of the instruments**. Never use wet pouches!

■ VOCABULARY & PRONUNCIATION GUIDE — CASE 10A

English Term	Spanish Translation	Simple Definition	Pronunciation Guide
Wicking	Capilaridad / Filtración por humedad	Process where liquid draws bacteria through porous paper packaging.	<i>WIK-ing</i>
Autoclave	Autoclave (Esterilizador)	Steam under pressure sterilizer unit.	<i>AW-toh-klav</i>
Drying Cycle	Ciclo de Secado	Essential final phase of autoclaving that removes moisture from instrument packs.	<i>DRY-ing SYE-kul</i>
Porous	Poroso	Having minute spaces or holes through which liquid or air may pass.	<i>POH-rus</i>
Standard Operating Procedures (SOPs)	Procedimientos Operativos Estandarizados	Written step-by-step instructions for office tasks and infection control.	<i>STAN-derd op-er-AY-ting proh-SEE-jers</i>

■ HELPFUL BOARD EXAM TIPS — CASE 10A

[!WARNING]

Sterilization Gold Rule:

Packages taken out of an autoclave while **wet** are considered **UNSTERILE** due to **wicking**. They must be re-packaged with fresh pouches and re-sterilized with a complete dry cycle!

■ ROLE PLAY 4: CHAPTER 10 — CASE B

Case Subject: Pedro, RDH (Disinfectant Contact Time, SDS & Infection Control Coordinator)

Characters & English Levels:

Pedro (Beginner English)*: Newly hired Registered Dental Hygienist (RDH).

Cindy (Intermediate English)*: Dental Assistant (hurried staff member).

Sarah (Advanced English)*: Infection Control Coordinator (ICC) & Lead RDH.

■ DIALOGUE SCRIPT: CASE 10B

Pedro (Beginner): Good afternoon! My name is Pedro. It is my first day at Oceanside Dental Group. Sarah, the Infection Control Coordinator, trained me this morning on the SOP manual. But I am wiping my operatory room between patients, and I forgot: how long must the surface disinfectant stay wet on the counter?

Cindy (Intermediate): Hi Pedro! Honestly, don't worry so much about Sarah's rules. Our schedule is super busy today! We don't have time to wait. Just wipe it and seat your patient right away!

Sarah (Advanced): Hold on, Cindy! Skipping disinfectant contact time puts our patients and staff at risk! Pedro, **Board Question #1** asks: How long should Pedro wait before seating his next patient after cleaning and disinfecting clinical contact surfaces?

Pedro (Beginner): Should I wait 1 minute? Or 10 minutes? Where do I check?

Cindy (Intermediate): Oh, I see... each disinfectant chemical has a different **contact time** (or dwell time) listed by the manufacturer on the container label!

Sarah (Advanced): That's right! You must **check the product's contact time on the container label** and allow the surface to remain wet for that full specified time! That answers **Board Question #1**. Now, **Board Question #2**: Pedro needs to find where the Dental Unit Water Line (DUWL) treatment tablets are stored. Where in the office should he look?

Pedro (Beginner): Should I ask Cindy again? Or look in a book?

Sarah (Advanced): Rather than relying on word-of-mouth, Pedro should **look in the Standard Operating Procedures (SOP) manual**! The SOP manual contains written instructions for water line maintenance and tablet locations.

Cindy (Intermediate): Ah, I understand now. And what about **Board Question #4**? If Pedro needs to check the chemical hazard information for our surface disinfectant spray, where is the **Safety Data Sheet (SDS)** located?

Sarah (Advanced): Safety Data Sheets (SDS) are legally required by OSHA to be kept in the **OSHA Manual / SDS Binder**, easily accessible to all employees!

Pedro (Beginner): Thank you, Sarah! So the OSHA manual has the SDS sheets, the SOP manual has office procedures, and the disinfectant label tells us the wet contact time!

Sarah (Advanced): Perfect summary, Pedro! That is why CDC guidelines recommend appointing a designated **Infection Control Coordinator (ICC)** to ensure office safety compliance!

■ VOCABULARY & PRONUNCIATION GUIDE — CASE 10B

English Term	Spanish Translation	Simple Definition	Pronunciation Guide
Contact Time (Dwell Time)	Tiempo de Contacto (Permanencia)	Required time a disinfectant surface must remain wet to kill target pathogens.	KON-takt TYME
Safety Data Sheet (SDS)	Hoja de Datos de Seguridad	Document detailing chemical hazards, handling, and first-aid measures.	SAYF-tee DAY-tah SHEET
Standard Operating Procedures (SOP)	Manual de Procedimientos (SOP)	Office manual listing exact step-by-step clinical protocols.	SOP MAN-yoo-ul
DUWL Treatment Tablets	Tabletas para Líneas de Agua	Antimicrobial tablets added to independent water reservoirs to inhibit biofilm.	D-U-W-L TREET-ment TAB-lets
Infection Control Coordinator (ICC)	Coordinador de Control de Infecciones	Designated staff member who monitors infection control compliance and training.	in-FEK-shun kon-TROHL koh-OR-dih-nay-ter

■ HELPFUL BOARD EXAM TIPS — CASE 10B

[!IMPORTANT]

OSHA & CDC Regulatory Distinctions:

- * **OSHA Manual:** Holds workplace safety rules, Bloodborne Pathogens Standards, and **Safety Data Sheets (SDS)**.
- * **SOP Manual:** Holds specific dental practice clinical protocols and product locations.
- * **Infection Control Coordinator (ICC):** Designated person recommended by CDC to oversee office training and compliance.

Viva Dental Prep — Clinical Role-Play Scripts for NBDHE Review